BASELINE → TREATMENT → FOLLOW-UP

Required and conditional checks across your treatment

Required checks are shown explicitly. electrocardiogram (ECG), additional urine tests and anticoagulant tests apply only when indicated. Scans and tumour markers follow your individual cancer plan; this page does not prescribe a universal surveillance interval. Confirm who books and reviews each test.

Check	Before starting	During treatment	Between visits	Review / change	After treatment
Blood counts / kidneys / liver	Required baseline blood count, kidney and liver tests	Before every cycle: each 14-day FOLFIRINOX cycle and each weekly capecitabine cycle	Earlier tests if unwell, dehydrated or toxicity develops	Recovery and results guide delay or dose adjustment	Follow unresolved abnormalities as agreed
dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) / fluoropyrimidine safety	DPD test before fluorouracil or capecitabine unless already tested	The result guides the starting plan; it does not exclude all severe toxicity	Unexpected severe early mouth sores, diarrhoea or illness needs urgent assessment	The team reviews suspected severe reactions before further treatment	Keep the result and any toxicity history in your records
Nerves / cold sensitivity	Record any pre-existing numbness or difficulty with daily tasks	Neuropathy assessment every cycle for this regimen	Report persistent tingling, trouble with buttons or walking; avoid cold triggers	Oxaliplatin may be changed separately after clinical review	Nerve symptoms can persist; arrange follow-up if unresolved
Heart assessment	Review cardiac history; baseline ECG if cardiac function is compromised	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
Radiotherapy / tablets	Confirm planning appointments and the phase-2 prescription	Review bowel, bladder, skin, hydration and tablet tolerance during chemoradiotherapy	Report worsening pelvic symptoms; follow the local radiation preparation plan	Do not restart held tablets or make up missed doses without instructions	Multidisciplinary restaging and surgery assessment are individual; follow late pelvic effects
Response / scans / tumour markers	Agree a baseline disease assessment and treatment aim	Review symptoms, function and clinical benefit	New symptoms may require earlier review	Scans and tumour markers follow the consultant-directed cancer plan	Agree next treatment or follow-up; there is no universal scan interval in this regimen

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00691 v4 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

FOLFIRINOX followed by chemoradiotherapy · rectal cancer: treatment and checks

Locally advanced rectal cancer

Phase 1: six FOLFIRINOX cycles, one every 14 days, each with a 46-hour fluorouracil pump. Phase 2: capecitabine twice daily on Days 1–5 of each seven-day week for five weeks, with radiotherapy. Your team supplies the separate radiation calendar.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Review / change	After treatment
Blood counts / kidneys / liver NCCP 00691	Required baseline blood count, kidney and liver tests	Before every cycle: each 14-day FOLFIRINOX cycle and each weekly capecitabine cycle	Earlier tests if unwell, dehydrated or toxicity develops	Recovery and results guide delay or dose adjustment	Follow unresolved abnormalities as agreed
DPD / fluoropyrimidine safety NCCP 00691	DPD test before fluorouracil or capecitabine unless already tested	The result guides the starting plan; it does not exclude all severe toxicity	Unexpected severe early mouth sores, diarrhoea or illness needs urgent assessment	The team reviews suspected severe reactions before further treatment	Keep the result and any toxicity history in your records
Nerves / cold sensitivity NCCP 00691	Record any pre-existing numbness or difficulty with daily tasks	Neuropathy assessment every cycle for this regimen	Report persistent tingling, trouble with buttons or walking; avoid cold triggers	Oxaliplatin may be changed separately after clinical review	Nerve symptoms can persist; arrange follow-up if unresolved

Care / check	Before starting	During treatment	Between visits	Review / change	After treatment
Heart assessment NCCP 00691	Review cardiac history; baseline ECG if cardiac function is compromised	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
Radiotherapy / tablets NCCP 00691	Confirm planning appointments and the phase-2 prescription	Review bowel, bladder, skin, hydration and tablet tolerance during chemoradiotherapy	Report worsening pelvic symptoms; follow the local radiation preparation plan	Do not restart held tablets or make up missed doses without instructions	Multidisciplinary restaging and surgery assessment are individual; follow late pelvic effects
Response / scans / tumour markers NCCP 00691	Agree a baseline disease assessment and treatment aim	Review symptoms, function and clinical benefit	New symptoms may require earlier review	Scans and tumour markers follow the consultant-directed cancer plan	Agree next treatment or follow-up; there is no universal scan interval in this regimen

What each check is for

1 Phase 1 · six FOLFIRINOX cycles

Day ward → home pump → review

This is a planned course before surgery assessment for locally advanced rectal cancer. Six FOLFIRINOX cycles are followed by capecitabine with pelvic radiotherapy. The team reviews response and the next part of your overall rectal-cancer plan.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

Sources: NCCP 00691

2 Phase 2 · five weeks with radiotherapy

Capecitabine tablets + pelvic radiotherapy

This is a planned course before surgery assessment for locally advanced rectal cancer. Six FOLFIRINOX cycles are followed by capecitabine with pelvic radiotherapy. The team reviews response and the next part of your overall rectal-cancer plan.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

Sources: NCCP 00691

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.33